

Submitted at www.regulations.gov

February 23, 2026

The Honorable Jim O'Neill
Office of the Deputy Secretary
Department of Health and Human Services
200 Independence Ave., SW
Washington, DC 20201

The Honorable Thomas Keane, MD, MBA
Office of the Assistant Secretary for Technology Policy/Office of the National
Coordinator for Health Information Technology
330 C St., SW
Washington, DC 20201

**Re: Comments Regarding the Accelerating Adoption and Use of Artificial
Intelligence as Part of Clinical Care: HHS Docket No. HHS-ONC-2026-0001**

Deputy Secretary O'Neill and Dr. Keane:

The Asian & Pacific Islander American Health Forum (APIAHF) writes to highlight critical considerations for Artificial Intelligence (AI) security and guardrails, workforce impacts and transparency as the Department of Health and Human Services considers AI adoption as part of clinical care.

The healthcare industry was amongst the first to explore AI applications and earlier adopters to deploy AI to scale – from clinical workflows to insurance approvals and other automated applications – utilizing AI at more than twice the rate of the broader economy.¹² AI has already transformed the healthcare landscape, and to disregard its impacts, both positive and negative, can only harm patients.

With a national network of over 300 community-based organizational partners in 44 states and territories, APIAHF is the nation's oldest national health advocacy organization dedicated to improving the health and well-being of over 26.4 million Asian Americans, Native Hawaiians and Pacific Islanders (AANHPIs) who would be affected by these technologies.

San Francisco Office:
461 Bush Street Suite 400
San Francisco, CA 94108
Main 415-954-9988

Washington DC Office:
1444 I Street NW Suite 700
Washington, DC 20005
Main 202-466-7772

www.apiahf.org

¹ Md Ashrafur Rahman, Evangelos Victoros, et al., *Impact of Artificial Intelligence (AI) Technology in Healthcare Sector: A Critical Evaluation of Both Sides of the Coin*, Clinical Pathology, (Jan. 22, 2024), <https://pmc.ncbi.nlm.nih.gov/articles/PMC10804900/>.

² Greg Yap, Derek Xiao, et al., *2025: The State of AI in Healthcare*, Menlo Ventures, (Oct. 21, 2025), <https://menlovcs.com/perspective/2025-the-state-of-ai-in-healthcare/>.

AI Security, Guardrails, and Public Trust

The promise of AI in clinical care cannot be realized without robust security controls and governance frameworks that protect sensitive health information and patient safety. AI systems handling protected health data must integrate cybersecurity best practices, privacy-by-design principles, and continuous validation protocols to guard against breaches, adversarial attacks, and systemic failure.

In addition to basic security measures, AI must consider health privacy protections including compliance with HIPAA guidelines.³ This means, any and all training for AI models must be in closed loop systems and have clear patient consent to drive improvement.

Without such protections, AI adoption risks eroding trust among patients, especially communities historically subject to discrimination in healthcare systems. Americans are concerned about how AI utilizes their data and how adoption of this data will affect their daily lives, including the quality of their healthcare treatment and coverage.^{4,5} In order to build and sustain trust, there must be robust oversight that includes quality standards and continuous monitoring and checkpoints – outside of industry, and in conjunction from trusted leaders within community.⁶

Equally important is transparency in model performance, limitations and validation.⁷ AI developers and healthcare organizations should publish performance metrics across diverse populations and explicitly disclose known limitations, so clinicians and patients can interpret outputs appropriately. This level of transparency enables informed decision-making, supports accountability, and guards against hidden algorithmic failures that could disproportionately harm marginalized groups.

Real-World Challenges and Risks

Data is the most critical component of AI, and quality data is essential to make AI work for all Americans.⁸ Accurate and representative data collection and data disaggregation is critical to eliminating biases and further reducing clinical errors with AI in healthcare.

³ Pushpendra Jangid, Asha Meena, et al., *The Role of Artificial Intelligence in Safeguarding Patient Privacy in Healthcare Systems*, Journal of Pharmacy & BioAllied Sciences, (Jun. 18, 2025), <https://pmc.ncbi.nlm.nih.gov/articles/PMC12244842/>.

⁴ Fathom, *Rising AI Use, Rising Calls for Action*, (Dec. 2025), <https://fathom.org/pdf/fathom-insights-report-2025.pdf>.

⁵ Jana Gundlack, Carolin Thiel, et al., *Patient's Perceptions of Artificial Intelligence Acceptance, Challenges, and Use in Medical Care: Qualitative Study*, Journal of Medical Internet Research, (May 15, 2025), <https://pmc.ncbi.nlm.nih.gov/articles/PMC12123243/>.

⁶ Fathom, *supra* note 4.

⁷ Jana Fehr, Brian Citro, et al., *A trustworthy AI reality-check: the lack of transparency of artificial intelligence products in healthcare*, Frontiers in Digital Health, (Feb. 20, 2024), <https://pmc.ncbi.nlm.nih.gov/articles/PMC10919164/>.

⁸ Lora Mois Aroyo, Matt Lease, et al., *Data Excellence for AI: Why Should You Care*, ACM IX Interactions, (2022), <https://research.google/pubs/data-excellence-for-ai-why-should-you-care/>.

AI bias is not hypothetical. AI should improve, not amplify biases in clinical performance. Real clinical AI tools have shown disparate effects across different demographic groups:

- AI models exhibiting underdiagnosis bias in chest radiographs for under-served patient populations due to bias across multiple datasets, specifically for female patients, younger patients, Black and Hispanic patients, and patients with lower socioeconomic status (via insurance coverage). With patients who belonged in more than one of these categories, they were even more likely to experience underdiagnosis.⁹
- An AI algorithm that assigned similar risk scores to Black and white patients even when Black patients had worse health outcomes, including Black patients with greater chronic conditions.¹⁰ The result was that Black patients were less likely than equally sick white patients to be enrolled for extra care or health programs. The algorithm didn't use race as a predictor, but because Black patients historically received less medical spending for the same level of illness, the real-world bias toward Black patients resulted in under-identified care.
- AI models to detect skin cancers were trained predominantly using datasets of images of lighter skin, developing an inherent bias toward the dominant skin tone and lesion colors from the trainings.¹¹ This can lead to underdiagnosis or misdiagnosis of patients with darker skin.

This is not to say that AI algorithms are inherently biased. These examples underscore how insufficiently diverse training data, lack of representative validation, and opaque model design can reinforce existing health inequities, particularly in communities that have already faced bias in clinical settings, such as many AANHPI subgroups.¹²

According to the 2024 National Academies report which revisits the groundbreaking 2003 *Unequal Treatments* report; racial and ethnic inequities in health and healthcare persist, costing our nation millions in preventable deaths and hundreds of billions of dollars annually.¹³ One of the key factors is the lack of uniform or comprehensive data on race and ethnicity which prevents data-driven decision making in clinical settings.¹⁴

Race and ethnicity are not the only considerations for AI bias. A widely used AI model used to help physicians diagnose and treat sepsis earlier identified just 7 percent of patients with sepsis who did not receive timely treatment, but did not detect sepsis in 67 percent patients of those who did develop sepsis.¹⁵ The model was tested in only a few geographical settings and was not correctly “calibrated” for

⁹ Laleh Seyyed-Kalantari, Haoran Zhang, et al., *Underdiagnosis bias of artificial intelligence algorithms applied to chest radiographs in under-served patient populations*, *Nature Medicine*, (Dec. 10, 2021), <https://www.nature.com/articles/s41591-021-01595-0>.

¹⁰ Ziad Obermeyer, Brian Powers, et al., *Dissecting racial bias in an algorithm used to manage the health of populations*, *Science*, (Oct. 25, 2019), <https://www.science.org/doi/10.1126/science.aax2342>.

¹¹ Navneet Melarkode, Kathiravan Srinivasan, et al., *AI-Powered Diagnosis of Skin Cancer: A Contemporary Review, Open Challenges and Future Research Directions*, *Cancers*, (Feb. 13, 2023), <https://pmc.ncbi.nlm.nih.gov/articles/PMC9953963/>.

¹² National Academies of Sciences, Engineering, and Medicine, *Ending Unequal Treatment: Strategies to Achieve Equitable Health Care and Optimal Health for All*, (2024), <https://www.nationalacademies.org/publications/27820>.

¹³ National Academies of Sciences, Engineering, and Medicine, *Ending Unequal Treatment: Strategies to Achieve Equitable Health Care and Optimal Health for All: Role of the Department of Health and Human Services*, (2024), https://nap.nationalacademies.org/resource/27820/Policy_Brief_Ending_Unequal_Treatment_HHS.pdf.

¹⁴ *Id.*

¹⁵ Liz Richardson, *Artificial Intelligence Can Improve Health Care—but Not Without Human Oversight*, *Pew*, (Dec. 16, 2021), <https://www.pew.org/en/research-and-analysis/articles/2021/12/16/artificial-intelligence-can-improve-health-care-but-not-without-human-oversight>.

other areas, showing that AI models are not one-size fits all. What works for certain populations does not mean it will work for others.

According to a 2020 study, out of 74 studies used to develop image-based diagnostic AI systems, there were only 16 states represented, with the majority of these studies relying on data from only California, New York, and Massachusetts.¹⁶ This means that rural states and rural communities were completely excluded – and their quality of care as diagnosed by AI suffers. This is especially concerning, given that AI is increasingly being deployed to supplement rural healthcare.¹⁷ Given the recent \$50 billion Rural Health Fund investment by the Centers for Medicare & Medicaid Services to transform and support rural health landscape, there must be intentional investment in rural health data to bridge gaps in care.¹⁸

To prevent AI from exacerbating disparities, developers and health systems must ensure diverse representation in data, bias audits and fairness oversight. AI research through intentional design and accountability must be core to HHS’s mission for implementation.

For the AANHPI community whose many subgroups span numerous languages, cultural practices and health profiles, standardized models trained on limited populations risk insensitive or inaccurate results. Disaggregated validation, robust bias mitigation methods and community-based monitoring must be core features of AI deployment frameworks.

Future Opportunities of AI in Health and Healthcare

Despite risks, AI can expand care access and quality in ways that directly benefit underserved communities, particularly when thoughtfully designed:

- Language access tools: AI-powered natural language processing and translation services can support real-time interpretation and documentation in clinical settings for limited English proficient (LEP) patients. This can improve communication, medication instruction comprehension, and treatment adherence across communities, especially for patients with complex medical needs.¹⁹ While many hospitals offer some sort of interpretation for LEP patients, many report that there is a shortage of available, qualified and timely translators, despite being mandated for large healthcare systems – leading to significant gaps in patient care.²⁰
- Culturally sensitive patient engagement: Generative AI can produce culturally distinct responses that helps tailor responses to cultural norms, beliefs, and health literacy levels, improving patient

¹⁶ Amit Kaushal, Russ Altman, et al., *Geographic Distribution of US Cohorts Used to Train Deep Learning Algorithms*, JAMA, (Sep. 2020), <https://jamanetwork.com/journals/jama/fullarticle/2770833>.

¹⁷ Kinalyne Perez, Daniela Wisniewski, et al., *Investigation into Application of AI and Telemedicine in Rural Communities: A Systematic Literature Review*, Healthcare, (Feb. 4, 2025), <https://pmc.ncbi.nlm.nih.gov/articles/PMC11816903>.

¹⁸ Centers for Medicare & Medicaid Services, *CMS Announces \$50 Billion in Awards to Strengthen Rural Health in All 50 States*, (Dec. 29, 2025), <https://www.cms.gov/newsroom/press-releases/cms-announces-50-billion-awards-strengthen-rural-health-all-50-states>.

¹⁹ Amelia K. Barwise, Susan Curtis, et al., *Using artificial intelligence to promote equitable care for inpatients with language barriers and complex medical needs: clinical stakeholder perspectives*, Journal of the American Medical Informatics Association, (Dec. 14, 2023), <https://pmc.ncbi.nlm.nih.gov/articles/PMC10873784/>.

²⁰ Elaine C. Khoong, Alicia Fernandez, *Addressing Gaps in Interpreter Use: Time for Implementation Science Informed Multi-Level Interventions*, Journal of General Internal Medicine, (May 4, 2021), <https://pmc.ncbi.nlm.nih.gov/articles/PMC8606497/>.

understanding and trust.²¹ Often, a lack of cultural sensitivity in clinical settings can cause further disconnect in patient-provider communication and trust.²²

- Bridging healthcare gaps in underserved settings: Like the rise of telehealth in rural areas, AI can assist in bridging healthcare gaps in underserved areas – especially in less-resourced regions that may not have access to certain technologies for early diagnosis and detection.²³

One example of AI models benefiting Asian Americans is the ability for early detection of stomach cancer through non-invasive screening methods. Stomach cancer is one of the deadliest cancers in the United States due to late diagnosis.²⁴ In the U.S., only 30 percent of patients who are diagnosed reach the five-year survival mark while in other countries with national screening programs, the survival rate is much higher from 60-70 percent.²⁵ Currently, medical societies do not recommend universal routine screening for stomach cancer in the U.S., because it is not “common in the U.S.” despite high incidence rates among certain groups.²⁶ Because there are no standard or routine screening tests recommended for stomach cancer, patients see higher insurance denials due to lack of “medical necessity.”²⁷

AI can create more efficient screening protocols by using noncontrast CT imaging which is considered a routine procedure and is more cost effective and less invasive than an endoscopy.²⁸ Additionally, CTs are more widely available and faster than endoscopies, making them a key tool, especially for populations that may not be able to quickly access care or insurance approvals.²⁹ This AI model, coined GRAPE, was created as a large-scale screening method to identify gastric cancer; a huge benefit for patients who have been denied from life-saving screenings due to cost or access.³⁰

²¹ Jackson G. Lu, Lesley Luyang Song, et al., *Cultural tendencies in generative AI*, Nature Human Behaviour, (Jun. 20, 2025), <https://www.nature.com/articles/s41562-025-02242-1>.

²² Saras Henderson, Maria Horne, et al., *Cultural competence in healthcare in the community: A concept analysis*, Health & Social Care in the Community, (Mar. 7, 2018), <https://onlinelibrary.wiley.com/doi/10.1111/hsc.12556>.

²³ Emma Gurevich, Basheer El Hassan, et al., *Equity within AI systems: What can health leaders expect?*, Healthcare Management Forum, (Oct. 13, 2022), <https://pmc.ncbi.nlm.nih.gov/articles/PMC9976641/>.

²⁴ Chul S. Hyun, Rong Wang, et al., *Gastric Adenocarcinoma in the United States: The Toll of Late Diagnosis and Racial and Geographic Disparities*, JCO Oncology Advances, (Oct. 27, 2025), <https://ascopubs.org/doi/10.1200/OA-25-00063>.

²⁵ Emily Moskal, *Stomach cancer hits Asian populations harder*, Stanford Medicine, (Dec. 16, 2022), <https://med.stanford.edu/news/insights/2022/12/stomach-cancer-asians.html>.

²⁶ Rachel Martin, *New Study Highlights Need for Coordinated Approach to Stomach Cancer Screening and Prevention*, Yale School of Medicine, (Oct. 27, 2025), <https://medicine.yale.edu/news-article/new-study-stomach-cancer-screening-and-prevention/>.

²⁷ Gretchen Morgenson, *‘Would he have lived?’ When insurance companies deny cancer care to patients*, NBC News, (Dec. 27, 2024), <https://www.nbcnews.com/news/investigations/-lived-health-insurance-companies-deny-cancer-care-patients-rcna182611>.

²⁸ Can Hu, Yingda Xia, et al., *AI-based large-scale screening of gastric cancer from noncontrast CT imaging*, Nature Medicine, (Jun. 24, 2025), <https://www.nature.com/articles/s41591-025-03785-6>.

²⁹ National Cancer Institute, *Computed Tomography (CT) Scans and Cancer*, <https://www.cancer.gov/about-cancer/diagnosis-staging/ct-scans-fact-sheet> (last visited Feb. 20, 2026).

³⁰ Hu, *supra* note 28.

Conclusion

AI holds transformative potential for clinical care, but only if adopted with strong guardrails, transparent evaluation, and equity-centered governance. Federal strategies should:

1. Mandate security, explainability and performance transparency across patient demographics.
2. Incentivize inclusive data practices, bias audits and community accountability boards to ensure equitable outcomes.
3. Prioritize tools that expand access and language-sensitive care for underserved and diverse populations.

By embedding these principles into AI adoption and oversight policies, HHS can safeguard patient welfare, uphold public trust, and foster innovation that enhances clinical care for all Americans.

For more information, please contact the APIAHF policy team (policy@apiahf.org). Thank you for your consideration of our comment.

Respectfully submitted,

Asian & Pacific Islander American Health Forum (APIAHF)